

Letter of instruction — psychiatric report

Dr Ravikumar Bangalore Krishnaiah, Consultant Psychiatrist, Mind and Memory Service
From: Cory Lea Shepherd · WC/2024/227, Queensland Industrial Relations Commission · [DATE]

PART A — THE ATTACHMENTS

Four documents accompany this letter. **Three of the four were written by the other parties to my matter — the Workers' Compensation Regulator, and my employer.**

No.	Document	Author	Status
1	Response of the Workers' Compensation Regulator , signed 18 February 2026 — a document in which the Regulator states its position, item by item, on a list of factual matters.	The Regulator	Assumed fact
2	Letter of the Chief Executive , Metro South Hospital and Health Service, 5 June 2026 (ref K-LM26/729), to the Commission.	The employer's Chief Executive	Assumed fact
3	Role description — Administration Officer, Switchboard Services (AO3), Logan Hospital.	The employer	Assumed fact
4	The clinical record — your own file for me from 24 October 2024; my general-practice records (Our Medical Ashmore); the workers' compensation medical certificates; my certificate of capacity of 3 July 2026.	Clinicians	Clinical record (not assumed fact)

Also enclosed, and not evidence: a **schedule of assumed facts**, which identifies by source and paragraph the facts referred to in Part B and records what the Regulator does *not* accept; and a copy of the **Notice of Non-Party Disclosure** served on your practice in this proceeding. **The four attachments themselves are provided complete and unmarked.**

PART B — THE BASIS ON WHICH I ASK YOU TO WRITE

I ask you to assume, for the purpose of your opinion, **the facts recorded in Attachments 1 to 3**. Attachment 1 records facts accepted by the Workers' Compensation Regulator; Attachments 2 and 3 are the employer's own documents.

The schedule also records the matters the Regulator does not accept. Please do not treat any of those as established. They are set out so that you have the position complete rather than one side of it.

My own account of the workplace is provided as clinical context only, and is not the basis on which I ask you to reason. Where your history from me differs from any fact in Attachments 1 to 3, please say so expressly rather than resolve the difference. Attachment 4 is the clinical record, for your clinical review.

Please do not adopt, or treat as established, any conclusion, characterisation or finding of any other decision-maker. **Your opinion is yours alone.** I do not ask you to express any view on whether any management action was reasonable; that is a question for the Commission.

[DECIDE BEFORE SENDING] Was Review Decision 69983 in the bundle provided to the practice on 12 August 2026? **If yes**, the express direction not to adopt it — and the question whether his opinion would differ if he disregarded it — must be inserted here. **If no**, this paragraph is deleted and the document is not mentioned. Set REVIEW_DECISION_PROVIDED in the build script.

The sequence, from a source other than my account. The following dates are taken from the Regulator's amended List of Documents dated 14 August 2026, by its item number.

Item	Date	Document
46	1 July 2024	Text, WorkCover to me, noting the claim registered
7	1 July 2024 11 August 2024 8 September 2024	Workers' compensation medical certificates — Dr Peter Hawes
8	7 August 2024	Workers' compensation medical certificate — Dr Ki Pang

Item	Date	Document
2	13 September 2024	WorkCover Queensland reasons for decision
3	16 September 2024	Application for review
9	24 October 2024	Email, Dr Krishnaiah — noting injury and medication
4	24 October 2024	Review Unit reasons for decision
5	26 November 2024	Notice of Appeal
10	13 February 2025	Report of Mind and Memory Service
11	Various	Practice records — Our Medical Ashmore

The clinical records are the complete records held by the Regulator in this matter, provided in full and unedited — including entries unrelated to this matter and entries that may not assist me. I have not selected among them. **If any further records or other material would assist you, please say so and I will obtain them.**

PART B2 — WHAT IS ALREADY RECORDED, AND WHAT I AM NOT ASKING YOU TO ESTABLISH

Attachments 1 to 3 already record the nature of the work, what occurred, and what the employer says was and was not in place. **You are not asked to find any of that, and you are not asked to take any of it from me.**

Already recorded, by the Regulator or by my employer: the requirements of the position · that the accuracy of Switchboard information is critical to clinical handover and patient safety · the applicable minimum break between shifts · the break in fact rostered on 17 to 18 March 2024, and its description as human error · an earlier rostering error acknowledged by the Director in August 2023 · that a leave application was declined although the required attachments were in fact present · the payroll instruction of 3 May 2024 and the correction of 28 May 2024 · that emergency (MET) calls were recorded on the shifts of 17 to 18 March 2024 · that leave was taken on 19 March 2024 · that no fatigue risk assessment applied to the position, that fatigue risk management was implemented only after 30 June 2024, and that no consequential changes to operating procedures followed · and that the general-practice entry of 16 November 2023 appears in the records.

What remains, and what I do ask you, is the clinical question: whether exposure of that kind bears on the condition you have diagnosed.

PART C — THE MATTERS IN ISSUE

You have been served with a Notice of Non-Party Disclosure in this proceeding. That notice states the matters in issue as follows:

- “1. Did Mr Shepherd sustain a personal injury*
- 2. Did the personal injury arise out of or in the course of Mr Shepherd's employment*
- 3. Was Mr Shepherd's employment a significant contributing factor to the injury”*

I ask you to address those three matters, in that order. My employer has separately asked for medical information about my capacity for work; **those matters are taken up within matter 3 below**, so that one report answers both.

MATTER 1 — DID MR SHEPHERD SUSTAIN A PERSONAL INJURY?

1.1 The diagnosis; the classificatory framework applied and the code; the specific diagnostic criteria met and when each was met; and the date of onset as best the clinical record establishes it.

1.2 The differential diagnoses considered, and your reasons for including or excluding each.

1.3 My background, and how it bears on your opinion. I ask you to reason from the following rather than around it.

(a) The past-history entries relied on. The records include a referral letter of **16 May 2024 from Dr William Zhao to Dr Arash Amini** which lists, under “*Past Medical History*”: **26/10/2022 ADHD · 26/10/2022 Anxiety · 28/06/2023 Infection**. Those entries are relied on as showing a past medical history of anxiety and ADHD. Please state:

(i) whether entries of that kind are **a record of a diagnosis made at a consultation, or a history-list item carried forward** in a practice record; and whether **any diagnosis of an anxiety disorder** appears anywhere in the material;

(ii) what the **current medications recorded in that same letter** indicate about whether any psychiatric condition was being treated as at **16 May 2024**; and

(iii) whether attention deficit is **a longstanding neurodevelopmental condition rather than a psychiatric injury**, and whether a person with those features is **more, or less, vulnerable to injury where continuous shift work is worked without adequate recovery between shifts**, and under sustained high-volume concurrent demand of the kind described at Attachment 3.

(b) The entry of 16 November 2023. Dr Priyal De Silva Nanayakkara recorded at that consultation: “*Poor sleep. Shift work. Takes melatonin 2 mg to help to go to sleep, soem time 4 mg, but does not help much. Cannot work/ do shifts if he does not get a good sleep. Does not take them every day. No psychological illness such as depression/ pyscosis. mood good.*” — and prescribed melatonin and temazepam. What does that entry indicate about **(i)** the presence or absence of psychiatric illness at that date, and **(ii) sleep disturbance attributed to shift work**, seven months before the date of onset in this matter?

(c) Anxiety recorded in the material. Please identify **each place anxiety is recorded, by author and date**, and in each case whether it is recorded as a **presenting symptom or made as a diagnosis** — including the consultation of **28 June 2024** recorded by Dr Bogdan Slawinski, at which the presentation is recorded as “*wants melatonin, to help to sleep ... stress at work ... upset by people not following rules*” and the reason for visit as “*Anxiety*”.

(d) My function in the role. Dr Peter Hawes recorded on 1 July 2024 that I had “*been there 5 years*”. Over that period I worked continuous shift work in this position. Please state what the material shows as to **any psychiatric treatment, or any absence from work on psychological grounds**, in the years before 2024 — and **what that indicates about my level of function in this role** over that period. Please also say whether anything in the records bears on the date of onset.

(e) If there was a pre-existing condition or vulnerability, whether the matters at Attachments 1 to 3 **aggravated** it, and to what extent.

1.4 Whether any premorbid personality features bear on the diagnosis, and if so how — from your own current clinical assessment. I do not ask you to adopt or repeat any earlier characterisation.

1.5 A complete list of every document you reviewed and every person you saw, with the dates and duration of each examination.

MATTER 2 — DID THE INJURY ARISE OUT OF, OR IN THE COURSE OF, THE EMPLOYMENT?

2.1 The conditions of the work. Attachment 3 records that the position requires continuous shift work across the full 24-hour period, seven days a week; participation in the Emergency Response process, distributing emergency notifications in accordance with code procedures and “*strictly adhering to protocols and timeframes*”; “*maintaining call queues to minimum at all times*”; multitasking and operating under pressure with “*high volume call traffic*”; exercising judgement “*in situations where precedence have not been set and procedures not defined*”; and working with “*limited supervision*”. Attachment 1 records that the accuracy of Switchboard information is **critical to clinical handover and patient safety**. **From a clinical perspective, whether and how conditions of that kind — the level of demand, the degree of control, and the adequacy of support — can contribute to a condition of the kind you have diagnosed.**

2.2 Fatigue, and the rostering of 17 to 18 March 2024. Attachment 1 records that on those dates I was rostered to finish at 23:00 and to commence the next shift at 06:00 — **a break of 7 hours**. The applicable minimum is recorded as **10 hours, or 8 hours by written agreement**. A further document of the Regulator,

dated 13 May 2026, records that an 8-hour written agreement applied and describes the shift as separated *“by only a 7-hour break (rather than an 8-hour break) ... a result of human error”*. **On either figure the break was shorter than the minimum.** Attachment 2 records that emergency (MET) calls were recorded on those shifts, and that leave was taken on 19 March 2024. Attachment 2 also records that **no fatigue risk assessment applied to the position**, and that fatigue risk management was implemented at that Switchboard only after 30 June 2024.

What clinical significance, if any, do you attach to that sequence — and does fatigue arising from a shortened break resolve with the break itself, or can it carry forward?

2.3 The administrative sequence. Attachment 1 and the further document of 13 May 2026 also record: an email of the Director dated **7 August 2023** acknowledging *“a rostering error that was accidentally made ... with regards to night shifts”*; that a leave application for **20 to 27 February 2024** was declined for want of an attached statutory declaration when *“a review indicates that in fact, the attachments were present”*, described as *“human error”*; and that a payroll instruction to correct my shifts was issued on **3 May 2024** and the correction submitted on **28 May 2024**, with an email of 21 May 2024 in the interval.

From a clinical perspective, does a sequence of that kind — repeated administrative error affecting rostering, leave and pay, in a role for which no fatigue assessment applied — bear differently on the condition you have diagnosed than a single isolated event would? Please explain the mechanism, if any.

MATTER 3 — WAS THE EMPLOYMENT A SIGNIFICANT CONTRIBUTING FACTOR?

This matter has three parts, and I ask you to take them in the order given — the other factors and their timing first, then the interval and the mechanism, and your conclusion last, so that the conclusion rests on the two answers before it. The questions on capacity, restrictions and adjustments follow at 3.4 to 3.7.

3.1 Other factors, placed in time. Please identify any non-employment factors relevant to **(i)** onset at or about **18 June 2024** and **(ii)** the subsequent course. Your report of 13 February 2025 referred to multiple life stressors including relationship breakdown, job loss and bereavement.

Your clinical relationship with me began on 24 October 2024, and my partner attended that consultation with me. Each of the matters above arose while I was under your care and is recorded in your own file. I ask you:

(a) to place each factor in time **from your own records rather than by assumption**, and **to state for each whether it arose before or after 18 June 2024**;

(b) to state, for any factor arising after that date, **whether it is capable of explaining the onset**; and to state whether, **as at the date of your diagnosis**, my relationship was a protective factor or an adverse one; and

(c) for each factor, whether in your opinion it was **independent of the employment matters, or a consequence of them** — noting that your report of 13 February 2025 records that my pay was *“withheld or delayed for up to five months at a time, leading to significant financial stress and strain on his relationship”*.

3.2 The interval, and the mechanism. The rostering and administrative matters at 2.2 and 2.3 occurred between August 2023 and May 2024, and the date of onset in this matter is **18 June 2024**. Is that interval consistent with, or inconsistent with, your formulation — and if consistent, **by what mechanism?** Separately, what significance, if any, attaches to the interval between onset and first presentation, the first workers' compensation medical certificate in the List of Documents being dated 1 July 2024?

3.3 Causation — your conclusion. On the assumed facts, and having regard to your answers at 3.1 and 3.2, whether my employment was **a significant contributing factor** to the injury with onset on 18 June 2024. Please give your clinical reasoning, and distinguish between the cause of onset and any factors bearing only on the subsequent course. **I do not ask you to find that employment was the only cause, or the main one** — only whether, on the material, it was a significant contributing factor.

Capacity, restrictions and adjustments — the matters my employer has asked about. My certificate of capacity of 3 July 2026 certifies me fit for my substantive role with adjustments; I ask only that your

opinion be your own.

3.4 Prognosis and current capacity. Your prognosis; and, distinguishing **(a)** my capacity to perform the substantive role **with reasonable adjustments** from **(b)** any current incapacity. If there is current incapacity, its cause — the condition itself, the consequences of my having been away from the workplace since 3 July 2026 (a matter of my account, not of Attachments 1 to 3), or other factors.

3.5 The adjustments, in functional terms. The adjustments you consider clinically necessary, the clinical basis for each, and the anticipated duration and review date — addressing, among anything else you consider relevant: predictability of rostering; adequate recovery between shifts; sustained concurrent emergency-code load; hours of work; and reporting arrangements. Please also address, in functional terms:

- (a)** any tasks, situations or environments likely to **exacerbate** the condition;
- (b)** if you consider a restriction on **complaint handling** clinically necessary, what activities it encompasses — receiving, documenting, redirecting or resolving complaints, or all complaint-related interaction; and
- (c)** your report of 13 February 2025 records that my **working memory** is affected under stress — what that means for performance of the duties at Attachment 3, and the circumstances likely to give rise to it.

3.6 The requirements of the position. Attachment 3 describes the duties of the position and states that it is a continuous shift working role. Please address, in order:

- (a)** whether the adjustments at 3.5 are **clinically necessary at present** — that is, whether I am presently able to perform the duties without them. **If your answer is that I am not, please identify which duties and why**, so that the answer is not read as a general incapacity for the role;
- (b)** whether, **with those adjustments in place**, I am able to perform the duties described at Attachment 3;
- (c)** whether adjustments of that kind — predictability of rostering, minimum recovery intervals between shifts, limits on sustained concurrent emergency-code load, and hours of work — are **of a kind ordinarily provided by large employers, and in particular by health services operating 24-hour services**; and
- (d)** whether the adjustments are **permanent, or temporary with a review date**.

I note that the certificate of capacity of 3 July 2026 at Attachment 4 records the arrangement then in place as a “*continuation of existing arrangement ... worked and tolerated ... without deterioration.*” Please take that record into account in answering (b) and (c). **[CONFIRM the exact wording against the certificate before sending]**

3.7 Foreseeable risk. Whether exposure to the conditions described at Attachments 2 and 3, **without** the adjustments at 3.5 and in the absence of a fatigue risk assessment, presents a foreseeable risk to my health or safety; and if so, what controls you consider medically necessary to manage it.

PART D — WHAT I DO NOT ASK, AND THE FORM OF THE REPORT

I **do not ask you** to express a view on whether any management action was reasonable; on whether my employer is able to accommodate the adjustments you recommend; or on my capacity to work under any particular reporting line or with any particular person. **Those are workplace questions, not medical ones. If you are asked any of them, I ask that you say so rather than answer.**

As to form, I ask that the report:

- (a)** state your qualifications and specialty, and the dates and duration of each consultation on which your opinion is based;
- (b)** **reason from the assumed facts to your conclusions rather than assert conclusions**, and state where the material is insufficient for you to express an opinion rather than qualifying an opinion you would not otherwise give;
- (c)** **state expressly the material on which your opinion as to causation rests, and whether that opinion depends on my account of the workplace matters**;

(d) state whether anything in the clinical records provided is inconsistent with, or qualifies, the opinions you express — and if so, how you have taken it into account, so that the report addresses the whole of the material rather than part of it; and

(e) confirm that the opinions expressed are your own.

The report is prepared for use in proceedings in the Queensland Industrial Relations Commission and may be provided to my employer. Please address it to me.